Memorial General Hospital - Department of Inpatient Medicine

CLINICAL DOCUMENTATION — NARRATIVE SPECIALTY CONSULT NOTE V1

Patient: Jessica White | **MRN:** PAT-60017 | **DOB:** 1986-08-15 | **Gender:** Female | **Doc ID:** NARRATIVE_0017
Encounter Date: 2025-06-09 | **Discharge/Note Date:** 2025-06-12 | **Provider:** Dr. Sarah Jenkins, MD

REASON FOR CONSULTATION

Consult requested by primary medical team regarding refractory clinical optimization and therapeutic titration. Consultant team: Cardiovascular Medicine Consult Service.

CHRONOLOGICAL CLINICAL NARRATIVE

CHRONOLOGICAL CLINICAL NARRATIVE: We were asked to evaluate patient Jessica White, who was admitted three days ago with complex multisystem decompensation. On initial presentation, initial biomarker profiling revealed white blood cell count of 14.9 10*3/uL (HIGH). Subsequent evaluation demonstrated that serum platelet count was 244.9 10*3/uL. Prior outpatient regimens had proven insufficient to achieve therapeutic stability.

HOSPITAL COURSE & TRAJECTORY

HOSPITAL COURSE & TRAJECTORY: During inpatient specialty evaluation, the consultant team adjusted pharmacological therapy. The patient was initiated on Cephalexin 250 mg by mouth QID for 10 days. Following this intervention, trajectory assessment showed calcium of 7.1 mg/dL (LOW). Clinical response has been favorable with marked improvement in objective metabolic parameters.

FOCUSED PHARMACOTHERAPY RECOMMENDATIONS

FOCUSED PHARMACOTHERAPY RECOMMENDATIONS: The specialty team recommends the following maintenance regimen: continue Metoprolol 25 mg oral BID; and titrate Lisinopril 5 mg oral every morning. Avoid NSAIDs and nephrotoxic exposures.

CONSULTANT ASSESSMENT & SPECIALTY PLAN

The patient is stabilized and appropriate for transfer to the regular step-down floor. Re-evaluate laboratory markers in 48 hours. Thank you for this interesting consultation.